

Nose Injury - Female (Child)

Algorithm 1228 | Child (0-17 years) | Gender at birth: Female

Definition

- Qatar-localized UAT-only adult and pediatric pathway for nasal trauma, screening for major bleeding, septal haematoma, head/neck/eye injury and safeguarding; not approved for production.
- Child patient.
- Female.

Initial assessment

1. [OPEN_TEXT] How did the injury happen?
2. [DURATION] When did it happen?
3. [YES_NO] Any bleeding, and has it stopped?
4. [OPEN_TEXT] What is the exact age; is the patient pregnant or anticoagulated; and are there loss of consciousness, vomiting, vision/neck symptoms, clear fluid, internal swelling, deliberate injury or inconsistent history?
5. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?
6. [CAREGIVER_OBSERVATION] What has a safe parent or caregiver observed about alertness, breathing, pain, swelling, fever, eating or drinking, sleep, play, and normal activity?

Triage and redirect questions

1. Disposition level 100

Is there a button battery, paired magnets, breathing difficulty, choking, severe bleeding, marked distress, or reduced responsiveness associated with suspected material in the nose?

The foreign-body family is not generated. Treat these as an emergency and call Qatar 999; do not attempt removal.

2. Disposition level 70

Is other food, an object, or foreign material inside the nose the primary concern rather than an injury, without the emergency features above?

The foreign-body family is not generated. Arrange urgent in-person assessment through the Qatar route approved by governance (GOVERNANCE_REQUIRED); do not attempt removal.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are GOVERNANCE_REQUIRED.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway is GOVERNANCE_REQUIRED.

3. Disposition level 100

Is there a nosebleed that will not stop, a large open wound with something in it, clear watery fluid trickling from the nose, a severe headache with blurred or double vision, eye pain, neck pain or stiffness with tingling in the arms, a purple swelling inside the nose blocking breathing, vomiting, or loss of consciousness?

NHS.UK broken nose guidance lists these as Qatar 999/an emergency assessment service selected through the Qatar pathway approved by governance (GOVERNANCE_REQUIRED) criteria - some suggest a serious head injury, not just a nose fracture.

3. Disposition level 70

Is the nose crooked since the injury, has swelling not started to go down after 3 days, are painkillers not helping, is there breathing difficulty once swelling subsides, or are there regular nosebleeds or fever?

NHS.UK guidance recommends contacting the Qatar clinical-review pathway approved by governance (GOVERNANCE_REQUIRED) for these features.

3. Disposition level 70

Is this mild bruising or swelling with none of the features above?

A fracture, septal haematoma and safeguarding risk cannot be excluded remotely; this UAT pathway requires in-person assessment.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are GOVERNANCE_REQUIRED. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact reporting, consent, documentation, and information-sharing rules are GOVERNANCE_REQUIRED.
- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Emergency nose injury precautions

- Call Qatar 999 and do not drive. Keep the patient observed; if awake with isolated bleeding, sit forward and pinch the soft nose, but avoid unnecessary movement if head or neck injury is possible. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: bleeding will not stop; vomiting, worsening confusion or breathing difficulty.

Urgent nose injury review

- Arrange same-day medical review, especially for a crooked nose or persistent swelling.
- Call Back If: breathing difficulty develops; fever develops.

In-person nasal injury assessment

- Do not straighten or manipulate the nose. Arrange prompt in-person review through the Qatar pathway approved for UAT; medication requires age, weight, pregnancy, bleeding and interaction checks.
- Call Back If: bleeding restarts; blocked breathing, fever, worsening pain or deformity.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are GOVERNANCE_REQUIRED. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Qatar 999 and do not drive. Keep the patient observed; if awake with isolated bleeding, sit forward and pinch the soft nose, but avoid unnecessary movement if head or neck injury is possible. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.

References

- NHS.UK, "Broken nose", <https://www.nhs.uk/conditions/broken-nose/> (page last reviewed 17 August 2023)

Provenance

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